
7-Day Hormonal Reset

A practical protocol to rebalance cortisol, oestrogen, progesterone and thyroid function through lifestyle and nutrition.

Designed for healthcare workers navigating shift work, stress and hormonal disruption

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Introduction

Hormones are the body's chemical messaging system — regulating everything from energy, mood, metabolism, sleep, libido, immune function, and pain sensitivity. Shift work, chronic stress, disrupted sleep, and nutrient depletion — all endemic to nursing — create a perfect storm of hormonal dysregulation. Elevated cortisol from chronic stress suppresses progesterone, disrupts thyroid function, and promotes insulin resistance. Night shift work alone has been shown to reduce melatonin by up to 50% and significantly disrupt the hypothalamic-pituitary-adrenal (HPA) axis (Axelsson et al., 2005).

This 7-day protocol is not a medical treatment — always consult your GP or an endocrinologist for persistent symptoms. Rather, it is a structured week of targeted lifestyle, nutritional, and behavioural practices that collectively support hormonal equilibrium. The science is solid: even modest improvements in sleep, blood sugar regulation, and stress reduction can meaningfully shift cortisol curves, improve thyroid conversion, and restore cycle regularity within weeks.

Key hormones addressed in this protocol:

Hormone	Role	Disrupted by
Cortisol	Stress response, energy, inflammation	Shift work, chronic stress, poor sleep
Oestrogen	Mood, cognition, bone health, cycle	Xenoestrogens, gut dysbiosis, stress
Progesterone	Sleep, calm, cycle regulation	High cortisol ('cortisol steal')
Melatonin	Sleep-wake cycle, antioxidant	Night shift light exposure, screens
Insulin	Blood sugar regulation, energy	Irregular eating, processed foods, stress
Thyroid (T3/T4)	Metabolism, mood, temperature	Nutrient deficiency, stress, inflammation

The 7-Day Protocol

DAY
1

Cortisol & Blood Sugar Reset

Cortisol follows a diurnal rhythm — peaking 30–45 minutes after waking (the Cortisol Awakening Response, or CAR) and declining through the day. Shift work and irregular eating destroy this rhythm. Day 1 focuses on anchoring the CAR and stabilising blood sugar — the two most powerful levers for cortisol regulation. Research shows that protein-rich breakfasts eaten within 60 minutes of waking significantly blunt the cortisol spike and reduce afternoon energy crashes (Jakubowicz et al., 2013).

Practices:

- Expose yourself to bright light within 10 minutes of waking (natural light or a SAD lamp — 10,000 lux for 20 minutes).
- Eat a protein-rich breakfast within 60 minutes of waking: 25–30g protein (eggs, Greek yoghurt, smoked salmon, legumes).
- Avoid caffeine for the first 90 minutes after waking — allow cortisol to peak naturally first.
- Eat every 3–4 hours to prevent reactive hypoglycaemia. No skipped meals today.
- Journal: Rate your energy and mood at morning, midday, and evening (1–10). This is your hormonal baseline.

***Nurse Tip:** On night shift, your 'morning' is whenever you wake. Apply the same rules: protein first, light exposure (even just opening blinds), delay caffeine by 90 minutes.*

DAY
2

Sleep & Melatonin Optimisation

Melatonin is produced by the pineal gland in response to darkness and signals the body to prepare for sleep. Blue light from screens (and hospital ward lighting) suppresses melatonin production by up to 85% (Gooley et al., 2011). Chronic melatonin disruption in shift workers is associated with increased risk of metabolic syndrome, breast cancer, depression, and cardiovascular disease. Today builds the foundations of hormonal sleep hygiene.

Practices:

- Create a sleep 'blackout' environment: blackout curtains, eye mask, white noise machine or earplugs.
- Begin screen-free wind-down 60 minutes before bed. Use blue-light blocking glasses from 9 pm onwards.
- Take 0.5–3mg melatonin 30 minutes before desired sleep time (especially for post-night shift sleep). Consult your GP first.
- Keep your bedroom temperature between 16–18°C (60–65°F) — core body temperature must drop for sleep onset.
- Magnesium glycinate (200–400mg before bed) supports both sleep and progesterone production — discuss with your GP.

***Nurse Tip:** Post-night shift sleep is often the least restorative. Prioritise sleep duration over sleep timing — even 6 hours of true darkness is hormonal medicine.*

DAY
3

Gut Health & Oestrogen Metabolism

The 'estrobolome' — the collection of gut bacteria responsible for metabolising oestrogen — is increasingly recognised as a critical regulator of hormonal balance. Poor gut health causes oestrogen to be reabsorbed rather than excreted, contributing to oestrogen dominance. Conversely, adequate fibre intake promotes healthy oestrogen clearance. Antibiotics — frequently taken or administered in healthcare — significantly disrupt the estrobolome (Plottel & Blaser, 2011).

Practices:

- Eat 30g of dietary fibre today: vegetables, legumes, whole grains, ground flaxseed (1 tbsp daily is particularly effective for oestrogen metabolism).
- Add 1–2 servings of fermented foods: kefir, natural yoghurt, kimchi, sauerkraut, or kombucha.
- Reduce or eliminate alcohol for this week — alcohol directly impairs liver oestrogen detoxification.
- Consider a high-quality probiotic containing *Lactobacillus* and *Bifidobacterium* strains.
- Hydrate: aim for 2–2.5L of water daily. Add electrolytes if on a long shift.

***Nurse Tip:** ICU environments are high-stress — stress-induced gut permeability ('leaky gut') accelerates hormonal disruption. Feed your gut bacteria on every day off.*

**DAY
4****Movement for Hormonal Health**

Exercise is one of the most powerful hormonal regulators available. However, the type and intensity matter enormously. High-intensity exercise (HIIT) raises cortisol — beneficial when timed correctly but damaging when overlaid on an already elevated cortisol baseline. Research shows that moderate-intensity exercise (Zone 2 cardio: 60–70% max heart rate) for 30+ minutes improves insulin sensitivity, reduces cortisol, boosts DHEA, and supports thyroid function (Hackney, 2020). Strength training raises IGF-1 and testosterone, improving energy and body composition.

Practices:

- 30 minutes of Zone 2 cardio: brisk walking, light cycling, swimming — you should be able to hold a conversation.
- If energy allows: 2 sets of bodyweight strength exercises (squats, lunges, push-ups) — 10–15 reps each.
- Avoid HIIT today and all week if you're in a high-stress period or feeling exhausted — it will raise cortisol further.
- Stretch for 10 minutes in the evening. Yoga nidra (20-min guided recording) is particularly effective for HPA axis restoration.
- Walk outside for at least 20 minutes — natural light and gentle movement together are a powerful hormonal combination.

***Nurse Tip:** A 12-hour night shift is physical exercise. On shift days, gentle walking and stretching are enough. Save more intensive movement for your days off.*

**DAY
5****Nutrition Deep Dive: Key Nutrients for Hormones**

Specific micronutrient deficiencies directly impair hormone production and metabolism. Zinc is required for progesterone synthesis and immune function. Iodine and selenium are essential for thyroid hormone production and conversion (T4 → T3). Vitamin D functions as a hormone itself, regulating hundreds of genetic processes including insulin sensitivity and oestrogen metabolism. Omega-3 fatty acids reduce the inflammatory prostaglandins that drive painful periods and PMS (Rahbar et al., 2012). Nurses are at significant risk of deficiency due to irregular meals, high physiological stress, and frequent exposure to illness.

Practices:

- Prioritise zinc-rich foods: pumpkin seeds, beef, oysters, chickpeas, cashews.
- Eat 2 Brazil nuts daily for selenium (thyroid support) — do not exceed 4 nuts per day.
- Check your Vitamin D level with your GP. Most people in northern latitudes require supplementation (1000–4000 IU/day).
- Add omega-3 rich foods: oily fish (salmon, mackerel, sardines) 2–3 times this week, or take a high-quality fish oil supplement (2g EPA+DHA/day).
- Reduce refined sugar and ultra-processed foods for the week — these drive insulin spikes, inflammation, and cortisol.

***Nurse Tip:** Hospital cafeteria food is rarely hormone-supportive. Meal prepping on days off — even just boiled eggs, mixed nuts, and chopped veg — can transform your nutritional baseline on shift.*

DAY
6

Stress Response & Adrenal Support

The adrenal glands produce cortisol, DHEA, adrenaline, and aldosterone. Chronic activation of the HPA axis — as seen in prolonged nursing stress — depletes adrenal reserves and can result in a flattened cortisol curve, persistent fatigue, poor immune function, and hormonal cascade failure. Adaptogenic herbs have been shown in randomised controlled trials to modulate cortisol response and support adrenal function (Pratte et al., 2014). Today focuses on parasympathetic activation and adrenal nourishment.

Practices:

- Ashwagandha (*Withania somnifera*): 300–600mg KSM-66 extract — shown to reduce cortisol by up to 27.9% in RCTs. Consult GP before starting.
- Practice 4-7-8 breathing (4 counts in, hold 7, out 8) for 5 minutes: directly activates the vagus nerve and lowers cortisol.
- Schedule at least 2 hours of unstructured restorative time today — no obligations, no screens, no productivity.
- Eat regularly. Skipping meals spikes cortisol. Today, eat 3 balanced meals with protein, fat, and complex carbohydrate.
- Limit caffeine to before noon and to a maximum of 2 cups — caffeine raises cortisol and impairs adenosine clearance.

***Nurse Tip:** You cannot pour from an empty cup — and neither can your adrenals. Treating rest as non-negotiable is clinical practice, not self-indulgence.*

The circadian clock governs the timing of virtually every hormone in the body — including cortisol, melatonin, insulin, growth hormone, and sex hormones. Shift work desynchronises the central clock (suprachiasmatic nucleus) from peripheral clocks in every organ, creating 'circadian misalignment' — a state associated with metabolic syndrome, reproductive disruption, increased cancer risk, and cardiovascular disease (Czeisler & Gooley, 2007). Today consolidates the week's practices into a sustainable circadian anchoring strategy.

Practices:

- Set consistent sleep and wake times even on days off — this is the single most powerful circadian anchor.
- Time-restricted eating: eat all meals within a 10–12 hour window (e.g. 7am–7pm). This aligns metabolic hormones with daylight.
- Review Days 1–6: which practices had the most noticeable effect on your energy, mood, or sleep?
- Create your personalised 'Hormonal Reset Maintenance Plan' — select 3 daily non-negotiables from this week.
- Book a baseline hormone panel with your GP: cortisol, DHEA-S, thyroid (TSH, T3, T4), Vitamin D, ferritin, B12, sex hormones.

***Nurse Tip:** Hormonal rebalancing is not a one-week fix — it is a months-long process of consistent inputs. This week gave you the foundation. The work is in showing up for yourself every day.*

Quick Reference: The 7-Day Protocol

Day	Focus	Key Action
1	Cortisol & Blood Sugar	Protein breakfast, light exposure, no early caffeine
2	Sleep & Melatonin	Blackout room, screen-free wind-down, magnesium
3	Gut & Oestrogen	30g fibre, fermented foods, flaxseed, reduce alcohol
4	Movement	30 min Zone 2 cardio, yoga nidra
5	Key Nutrients	Zinc, selenium, Vitamin D, omega-3, reduce sugar
6	Adrenal Support	Ashwagandha, 4-7-8 breathing, 2hrs restorative rest
7	Circadian Reset	Consistent wake time, time-restricted eating, maintenance plan

***Medical Disclaimer:** This guide is for educational purposes only and does not constitute medical advice. Always consult a qualified healthcare professional before beginning any supplementation or if you have concerns about your hormonal health.*

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